

Clinical Decision Making of the Cognitively Impaired or Acute Psychotic Patient: Assessment of Capacity and Civil Commitment

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Do.
Make.
Heal.
Innovate.
Reinvent the Future.

1. Students will understand the general categories of civil commitment.
2. Students will know the difference between capacity and competency.
3. Students will become knowledgeable about due process and patient rights in for admitted psychiatric patients.
4. Students will learn the four components to determine clinical capacity in the cognitively impaired or actively psychotic patient.

Admitted Patients Rights

- Civil Right Retained and Protected
- Personal Rights
- Privacy and Confidentiality
- Correspondence, Telephone, Visitors
- Right to Quality Care
- Right to Object to treatment
- Restraints and Seclusion
- Surgery and other treatments-informed consent
- Research-Right to Refuse



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Civil Commitment:

Voluntary

VS.

Involuntary Admission



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Emergency Admissions

VS.

2 Physicians Certificate (NYS)

Arkansas Inpatient and outpatient commitment

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A person shall be eligible for involuntary admission if he or she is in such a mental condition as a result of mental illness, disease, or disorder that he or she poses a clear and present danger to himself or herself or others



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Commitment

vs.

Competency



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Right to Refuse

Medication

ECT



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RIVERS vs. KATZ

- **Lacks capacity**
- **Due to a mental illness**
- **Likely benefit**
- **No less restrictive treatment**



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Capacity Determination

- **Mental Illness: Psychosis**
- **Mental Retardation**
- **Cognitive Impairment: Dementia, Delirium, Acute Intoxication**



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Capacity Determination *(cont'd)*

- **Express preference or awareness:**

- **Factual understanding**
 - **Appreciate seriousness of the conditions**
-
- **Able to manipulate information in a rational fashion.**



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NYS Mental Hygiene Legal Service –
All patients must have “due process”
NYS Supreme Court-Hears all patient
request for discharge and medication
over objection.

Ten Myths About Capacity

1. Decision-making capacity = competency.
2. Against medical advice = lack of decision-making capacity.
3. There's no need to assess decision-making capacity unless a patient goes against medical advice.
4. Decision-making capacity is all or nothing.
5. Cognitive impairment = no decision-making capacity.
6. Lack of decision-making capacity is permanent.
7. Patients who have not been given relevant information about their condition can lack decision-making capacity.
8. All patients with certain psychiatric disorders lack decision-making capacity.
9. All institutionalized patients lack decision-making capacity.
10. Only psychiatrists and psychologists can assess decision-making capacity.

C R A M

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1. Communication of a stable choice
2. Relevant information is understood
3. Appreciation of the situation
4. Manipulation of information in rational manner

Lecture Feedback Form:

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<https://comresearchdata.nyit.edu/redcap/surveys/?s=HRCY448FWYXREL4R>